

Contaminated patient

Version 23

**Proforma and action cards
(ED CBRNe & HAZMAT pack) to guide
decisions about decontamination,
care coordination and disposition**

**Tear off and hand out the relevant
action cards as situation demands**

**NB: The full action card set for
multi-patient incidents is available
from the ED Major Incident Room**

**All staff involved in the incident
to sign the log on the back of
this sheet (box 6)**

Disclaimer:
This is a clinical template; clinicians
should always use judgment when
managing individual patients

Approved as pilot by ED Senior Management Team
on 17Dec2025 - Review due: Dec 2026

Patient details

Full
name

DoB

Unit
number

(use sticker if available)

① Are there symptoms?

☐ **Yes** - one or more of the below

Itching ☐
Burning ☐
Rash ☐
Eye symptoms ☐
Cough ☐
DiB ☐
Seizures ☐
Confusion ☐

☐ **No** - none of the above

② Implicated substance

Gas ☐
Liquid ☐
Powder ☐
Radiation ☐

Substance name (if known)

Fire service already involved ☐
Fire service **NOT** involved ☐

Red-call script (ER nurse)

1. **During the call** look at boxes 1-4, ask the relevant questions and record information
2. **Before ending call**, ask crew to wait outside to be met by ED staff
3. **After ending the call**, immediately tannoy 'NIC and EPIC to decontamination access point STAT'
4. Hand proforma to NIC/EPIC - will meet you at decon room exterior door

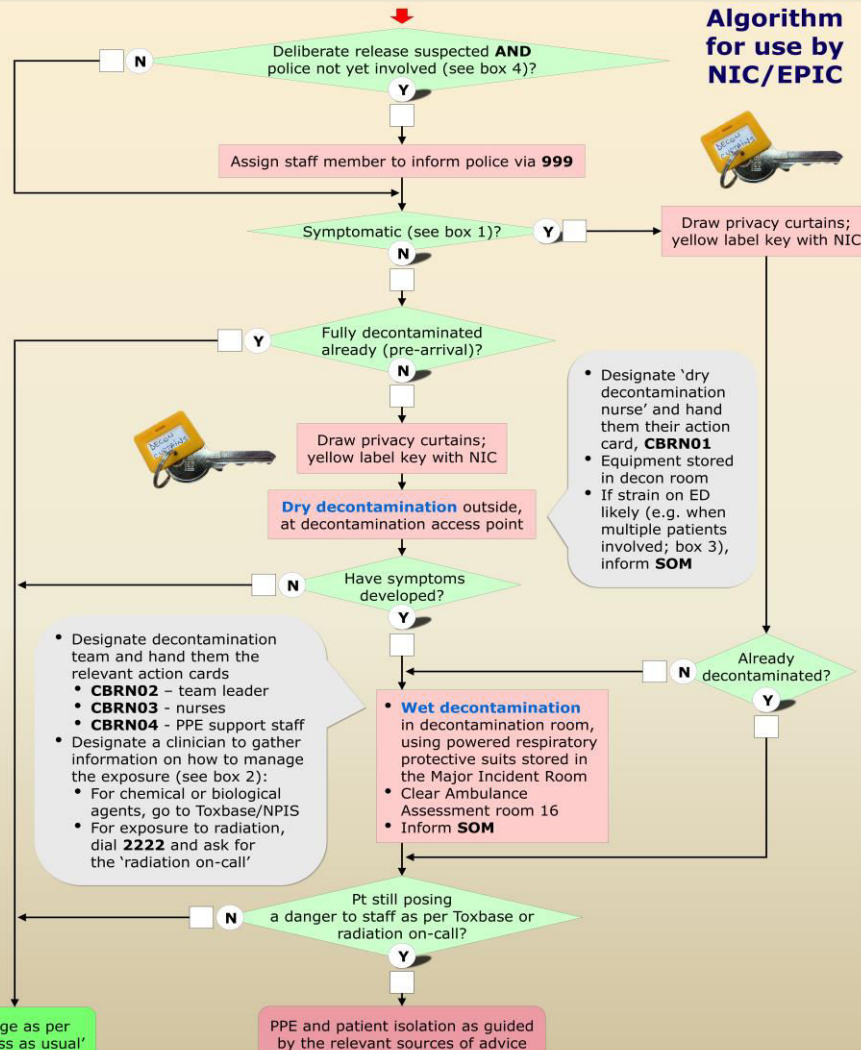
Walk-in script (Adult or Children's VAC nurse)

1. Immediately ask patient to step back outside and wait for you there
2. Ask a front door colleague to immediately tannoy 'NIC and EPIC to decontamination access point STAT'
3. Meet patient outside and signal everyone else to stand clear of them
4. Walking behind them, guide patient to the [decontamination access point](#) (for CED, scan QR code for a brief video of the required route if needed)
5. Look at boxes 1-4, ask the relevant questions and record information
6. Hand proforma to NIC/EPIC - will meet you at decon room exterior door

From CED



**Algorithm
for use by
NIC/EPIC**



Record a Datix and email EPRR team at uhl-tr.spoc@nhs.net with a brief description of the incident.
If powered respiratory protective suits (PRPS) were used, also complete the actions listed in box 5.

③ Multiple casualties?

☐ **Yes** how many?

☐ **No**

④ Release deliberate?

☐ **Yes** - CBRNe incident

Police already involved ☐
Police not yet informed ☐

Crime number

☐ **No** - HAZMAT incident

⑤ PRPS disposal checklist

use box 7 to record the details below

- ☐ Record a final pH reading
- ☐ If radiation incident, record both background and suit RAM GENE-1 readings
- ☐ Box up the suit and record the name of the staff member wearing it and the box number. **NB: Matching suit with its original box is unimportant as long as the suit and box size are the same.**
- ☐ Place box in designated spot outside of the decontamination room (below the poster 'No Public Access STAFF ONLY')
- ☐ Notify ED porters to take the box away (will follow their own specific process)
- ☐ Email uhl-tr.spoc@nhs.net (the EPRR team), copying and pasting the text below and adding the relevant details

Dear EPRR team, we used this PRPS:

Box number

Worn by [full name]

Final pH

[also, if radiation involved:]

RAM GENE-1 readings

- Background

- Suit

Full
name

DoB

Unit
number

(use sticker if available)

⑥ Staff contact log

Role	Print Name	Grade	Time in
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Red call taken by/
VAC nurse

ED NIC

EPIC

Dry decon

Wet decon team leader

Senior operational manager (SOM)

Radiation incidents only

Box number	Suit worn by	pH	RAM GENE-1	RAM GENE-1
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Box number

Suit worn by

pH

RAM GENE-1 background

RAM GENE-1 suit

**University Hospitals of Leicester
CBRNe & HazMat Plan**

CBRN01

**DRY Decontamination –
Decontamination Nurse**

Names:

Accountable To: ED Decontamination Lead

Base Location: One nurse in the Ambulance Bay outside the decontamination room, One nurse inside the decontamination room

**CHEMICAL OR BIOLOGICAL SUBSTANCE
DRY DECONTAMINATION IS THE DEFAULT DECONTAMINATION PROCESS**

Roles: To ensure that casualties are prepared to be decontaminated safely and are suitably prioritised. You will be responsible for the management of all patients prior to decontamination.

CORE ACTIONS: Remember that the action card listings are not exhaustive

TIME & INITIALS

Immediate actions upon activation of this plan

1. Locate decontamination equipment in the decontamination room and consider using modesty panels not only to preserve dignity, but to also use the visual cards on the panels to better explain the decontamination process.
2. Put on a long sleeved gown, gloves, FFP3 mask and eye protection.
3. Assess casualties for any signs or symptoms of exposure to caustic or irritant substances (for example, redness, itching and burning of the eyes or skin)

IF THEY DISPLAY ANY OF THE SIGNS ABOVE, PROCEED WITH WET DECONTAMINATION PROTOCOLS.
4. Ensure you do not come in direct contact with casualties – Keep 2 meters distance where possible.
5. Prepare the decontamination tools (available in the trolley in the decontamination room): the gowns, blankets, the clinical waste orange bags, the blue towels, patient property bags, pens, sanitary towels.
6. Ensure the privacy screens are unfolded to create an area with some privacy for decontamination. Use the pictures on the screens to better communicate instructions to everyone who needs dry decontamination.
7. Patients will undress outside the decontamination room. The VAC nurse will supervise. Do not allow items to be removed over the top of their head – cut off if necessary. (use scissors but DO NOT throw them at the patients)
8. If there are any patients that refuse decontamination due to modesty concerns, make use of the ORANGE – DISROBE gowns.
9. The VAC nurse will then ask the patients to blot and gently rub their own skin with blue towelling, starting with the face, head and neck and moving down and away from the body. The casualty should blow their nose and spit out. The rubbing and blotting should not be too aggressive, or it could drive contamination further into the skin. Patients will place the used blue towel in their orange bag.

10. Ask patients to put their valuables in a patient property bag and ask them to write their names on the bag. Depending on the contaminant they may receive the bag after decontamination.	
11. Once the patients have undressed, ask them to put their clothing in an orange bag which is double bagged and labelled with the patient's name using a marker pen.	
12. Invite patient to walk forward, leaving their orange bag and valuables behind.	
13. Offer patients blankets to keep themselves warm, especially, if wet decontamination will be required post dry decontamination.	
14. The decontamination nurses will then seal, label and double bag the waste orange bags and put the sealed bags in a pile outside the decontamination room against the partition.	
15. The decontamination nurses will then put all the floor covers in an orange bag, seal double bag and add to the waste pile outside. Fresh floor cover and blue towel is then put out ready for the next batch of patients.	
16. Patient will book/check in in at the Ambulance Reception Desk.	
17. Initial Assessment Nurse and Doctor will assess patient.	
18. Once all self-presenters have been decontaminated, consider, and discuss own decontamination and agree with Nurse in Charge or EPIC decision.	
Actions upon stand-down of this plan:	
The decision to stand-down this plan will be communicated via Section 5.11 of the Incident Response Plan	
1. Attend the hot debrief.	
2. Keep all paperwork, notes, records and logs associated with the incident and at the end of the incident, submit these to the EPRR Team	
Role Continuity	
1. Only undertake this role if you are fit and well, and adhere to working time limits	
2. If the incident extends past your shift, arrange a suitable replacement and allow time for handover	
Communications	
Any incident will likely generate a lot of interest from both the public and the media. In the event of receiving calls or queries asking for information please use one of the following responses: Other Patients: An event has taken place and colleagues are in the process of responding to it. Unfortunately, we do not have any further information at this point in time. The Trust will release a public statement as soon as possible and this will be made available on the UHL website. General Public: We have been made aware that an event has taken place, but we are unable to release any information at this point in time. However, the Trust will be releasing a public statement as soon as possible and this will be made available on the UHL website. Patient Relative (over the phone): We have been made aware that an event has taken place, however we are unable to release any information at this point in time. I am unable to give out any patient details over the telephone. If you are concerned about somebody involved in an incident, please let me have contact details of the person/people you are concerned about, and somebody will call you back at the earliest opportunity.	

**University Hospitals of Leicester
CBRNe & HazMat Plan**

CBRN02

Decontamination Team Leader

Names:

Accountable To: EPIC and Nurse in Charge

Base Location: Ambulance Assessment Bay

CHEMICAL, BIOLOGICAL, RADIOLOGICAL, NUCLEAR or UNKNOWN SUBSTANCE

Roles: This role will be undertaken by a band 6 or above. You will be responsible for coordinating all the activities within the decontamination area.

CORE ACTIONS: Remember that the action card listings are not exhaustive

TIME & INITIALS

Immediate actions upon activation of this plan

1. Liaise with EPIC or Nurse in Charge and identify decontamination team members. Brief them of the situation then ensure they are on standby.
2. Support the allocation of roles during and post initial briefing. Some roles may require multiple staff to undertake. Staff wearing PRPS should not be involved in setting up the decontamination area: their role is to don suits on and manage patients who require decontamination. Ensure a nurse is allocated to time the decontamination process and that each staff spends no more than 60 minutes in the PRPS suit.
3. Communicate with VAC nurse while keeping your distance and ensure all equipment needed is brought into the decontamination room. You may need to liaise with the Majors Coordinator to move patients out of the Assessment Bay.
4. In the event of Radiation/Nuclear contamination – ensure RAMGENE is available for the decontamination nurses.
5. Coordinate the decontamination process.
6. Consider using modesty panels not only to preserve dignity, but to also use the visual cards on the panels to better explain the decontamination process.
7. Ensure cubicles 15,16 and 17 can be made easily available. Post decontamination cubicle 16 will be used for regrouping the self-presenters.
8. Evaluate at the number of Decontamination Nurses and request additional staff from the ED Command Team (Nurse in Charge, EPIC and CMG Head of Operations) if required.
9. Direct patients to re-dress and book in at the Ambulance Reception Desk.
10. Allocate patients to an Assessment Bay cubicle available for the Initial Assessment Nurse and Doctor to see.

Actions upon stand-down of this plan:

The decision to stand-down this plan will be communicated via Section 5.11 of the Incident Response Plan

1. Attend the hot debrief.
2. Keep all paperwork, notes, records and logs associated with the incident and at the end of the incident, submit these to the **EPRR Team**
3. Clear area and ensure it is returned to pre-event/incident condition

Role Continuity

- | | |
|--|--|
| 1. Only undertake this role if you are fit and well, and adhere to working time limits | |
| 2. If the incident extends past your shift, arrange a suitable replacement and allow time for handover | |

Communications

Any incident will likely generate a lot of interest from both the public and the media. In the event of receiving calls or queries asking for information please use one of the following responses:

Other Patients: An event has taken place and colleagues are in the process of responding to it. Unfortunately, we do not have any further information at this point in time. The Trust will release a public statement as soon as possible and this will be made available on the UHL website.

General Public: We have been made aware that an event has taken place, but we are unable to release any information at this point in time. However, the Trust will be releasing a public statement as soon as possible and this will be made available on the UHL website.

Patient Relative (over the phone): We have been made aware that an event has taken place, however we are unable to release any information at this point in time. I am unable to give out any patient details over the telephone. If you are concerned about somebody involved in an incident, please let me have contact details of the person/people you are concerned about, and somebody will call you back at the earliest opportunity.

**University Hospitals of Leicester
CBRNe & HazMat Plan**

CBRN03

WET Decontamination – Decon Nurse	Names:
Accountable To: ED Decontamination Lead	Base Location: UHL Tactical Incident Coordination Centre

CHEMICAL, BIOLOGICAL, RADIOLOGICAL, NUCLEAR or UNKNOWN SUBSTANCE

Roles: To ensure that casualties are prepared to be decontaminated safely and are suitably prioritised. You will be responsible for the management of all patients prior to decontamination.

CORE ACTIONS: Remember that the action card listings are not exhaustive	TIME & INITIALS
Immediate actions upon activation of this plan	
1. Before each group of casualties, the decontamination room will have a roll of blue towel placed for each patient.	
2. Prepare the decontamination tools (available in the trolley in the decontamination room): the gowns, blankets, the clinical waste orange bags, the blue towels, patient property bags, pens, sanitary towels.	
3. Don PRPS Suits.	
4. Consider use of RAMGENE to filter contaminated casualties pre wet decontamination.	
5. Ensure the privacy screens are unfolded to create an area with some privacy for decontamination. Use the pictures on the screens to better communicate instructions to everyone who needs decontamination.	
6. Patients will undress outside the decontamination room. The VAC nurse will supervise. Do not allow items to be removed over the top of their head – cut off if necessary.	
7. Casualties will need to undress down to their underwear and will be given a clinical waste orange bag to add all their clothes – please direct them to touch the outside of them as little as possible. Ask patients to add any valuables in the patient propriety bags with white labels and add their name to the label.	
8. If there are any patients that refuse decontamination due to modesty concerns, make use of the ORANGE – DISROBE gowns.	
9. Offer casualties awaiting decontamination a gown, socks, mask, sanitary towels (if needed) and a blanket to keep themselves warm (if needed).	
10. These blankets and gowns should be added to an orange bag prior to the patient entering the decontamination room, then double bagged and added to the waste pile outside the decontamination room.	
11. Casualties will be decontaminated using the Rinse-Wipe-Rinse method. It is important to try and limit the duration of decontamination to about a minute (alkalis might take much longer). The duration of the wet decontamination can be confirmed by EPIC with NPIS.	

12. Ask the patient to use the handheld shower starting with the head tilted back, rinsing the face and then the hair (no rinsing hair leaning forwards as this contaminates the face). Then showering down the rest of the body.	
13. Patients will then dry themselves and drop their towel on the floor in the decontamination room	
14. IF this is a suspected or confirmed radiological/nuclear event – use the RAMGENE to identify any potential contaminants. If any discovered, request the patient to go through the wet decontamination procedure again.	
15. Post decontamination, patients may step into the 'green' areas outside the decontamination room and dress into scrubs or pyjamas (if it is clear they will require admission).	
16. Patients will book/check in in at the Ambulance Reception Desk.	
17. The decontamination nurses will then pick up the capes and towels and put them in orange bags, seal, label with the patient's name using a black marker pen and double bag the waste orange bags and put the sealed bags in a pile outside the decontamination room against the partition.	
18. Ensure the room is ready for the next set of casualties that need wet decontamination.	
19. If all casualties have been decontaminated OR you have been in the suit close to 60 min, prepare for self-decontamination.	
20. Post wet decontamination, request checking Ph testing of the PRPS suits from Decontamination Leader.	
21. IF Ph neutral (7), disrobe suits in the blue boxes and ensure these are placed outside the decontamination room. These suits will need recertifying.	
22. IF Ph showing alkaline (>7) or acidic (<7), disrobe suits in the blue boxes and use a marker to clearly mark the box. These suits will need disposing of.	
Actions upon stand-down of this plan:	
The decision to stand-down this plan will be communicated via Section 5.11 of the Incident Response Plan	
1. Attend Debrief	
2. Keep all paperwork, notes, records and logs associated with the incident and at the end of the incident, submit these to the EPRR Team	
Role Continuity	
1. Only undertake this role if you are fit and well, and adhere to working time limits	
2. If the incident extends past your shift, arrange a suitable replacement and allow time for handover	
Communications	
Any incident will likely generate a lot of interest from both the public and the media. In the event of receiving calls or queries asking for information please use one of the following responses:	
Other Patients: An event has taken place and colleagues are in the process of responding to it. Unfortunately, we do not have any further information at this point in time. The Trust will release a public statement as soon as possible and this will be made available on the UHL website. General Public: We have been made aware that an event has taken place, but we are unable to release any information at this point in time. However, the Trust will be releasing a public statement as soon as possible and this will be made available on the UHL website. Patient Relative (over the phone): We have been made aware that an event has taken place, however we are unable to release any information at this point in time. I am unable to give out any patient details over the telephone. If you are concerned about somebody involved in an incident, please let me have contact details of the person/people you are concerned about, and somebody will call you back at the earliest opportunity.	

Procedure for using the RAM GENE Monitors

1. Do a background check away from patients
 - a. Turn on the unit – leave cap on – Check it is in dose rate mode, if the dose-rate is above 1 μ Sv/h contact medical physics
 - b. Take the cap off – check in CPS mode (light lit) if not check the sensor under the rim of the detector head is not covered if over 2 cps contact medical physics.
2. Scan patient
 - a. Put the cap back on and point at patient from a distance. (ensure you are not covering the sensor with your hand)
 - b. Walk towards the patient and if at any time it alarms – stop – you should consider this the extent of the safe distance to maintain from the patient. If it alarms stop and jump to stage 3.
 - c. Take the cap off and ensure the cps light is illuminated – if it isn't check that you are not covering the sensor under the rim of the detector head.
 - d. Scan the patient slowly at a distance of 1cm checking their body (use a tongue depressor to ensure you are not touching the patient). If any areas alarm/over 10% of background reading then contamination is present, inform the timing nurse to make a note. Stop and jump to stage 3. e. If no alarms sound jump to stage 4
3. Decontamination
 - a. Instruct the patient to decontaminate; notify timing nurse/triage nurse of findings
 - b. If after 2 cycles of decontamination and alarms still sound assume internal contamination. No need to repeat decontamination – notify medical physics and medical team.
4. If no alarms sound, consider the patient safe.

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**University Hospitals of Leicester
CBRNe & HazMat Plan**

CBRN04

PPE Support

Names:

Accountable To: ED Decontamination Lead

Base Location: Assessment Zone

CHEMICAL, BIOLOGICAL, RADIOLOGICAL, NUCLEAR or UNKNOWN SUBSTANCE

Roles: This role will be undertaken by an HCA who will assist colleagues with donning and doffing PPE. Work with the Timing Nurse to ensure adequate staffing as well as donning/doffing is maintained throughout the response.

CORE ACTIONS: Remember that the action card listings are not exhaustive

TIME & INITIALS

Immediate actions upon activation of this plan

1. Ensure you are wearing your PPE: disposable scrubs, gloves (double gloves if wet decontamination is required), a face mask and surgical boots.

2. IF DRY Decontamination, ASSIST DECON NURSE to put on:

- 1st pair of gloves
- FFP3 mask/respirator
- surgical hood
- gown or one piece coverall
- plastic apron over gown
- 2nd pair of gloves over cuffs of gown
- disposable visor

3. Check PPE ensuring that there is no exposed skin visible once all PPE is worn.

4. IF DRY Decontamination, ASSIST DECON NURSE with DOFFING:

5. Ensure that there is a large yellow medi-bin available for disposing of all used PPE

6. Decontamination nurse to remove plastic apron

7. PPE Support staff to apply hand sanitiser on to Decontamination Nurse's gloved hands. Clean gloves

8. Decontamination Nurse to remove gloves

9. PPE Support staff to apply hand sanitiser on to Decontamination Nurse's hands. Clean hands

10. Decontamination Nurse to remove gown turning inside out and rolling up. Be careful not to touch outside of gown or coverall.

11. PPE Support staff to apply hand sanitiser on to staff members hands. Clean hands

12. Decontamination Nurse to remove disposable visor

13. PPE Support staff to apply hand sanitiser on to staff members hands. Clean hands.

14. Decontamination Nurse to remove mask or respirator

15. Decon Nurse to clean hands with soap and water and then use alcohol hand sanitiser

16. IF Wet Decontamination, ASSIST DECONTAMINATION NURSES to put on the PRPS Suits by reading out loud and following the below instructions:	
17. Sit the staff member down on a chair and help them into the suit one leg at a time	
18. Ask the staff member to stand up and walk forward – caution the chair is likely to tip over	
19. Strap the Air Filter Unit (AFU) around the waist using the red straps and buckle	
20. Put on the cotton white gloves	
21. Place both arms into the suit	
22. When ready to be deployed buddy to reach in and turn the AFU on (wearers right hand side)	
23. Wearer to duck forward as PPE Support pulls the hood over the head and face – there will be a brief period where the visor will be in contact with the skin, this will resolve once the suit is sealed.	
24. Wearer to put both arms up and out to the side (star jump pose) and PPE Support to zip up. Make sure the zip is completely sealed.	
25. Check the suit for any damage – check seams and gloves	
26. Check the visor – no scratches or scuffs	
27. Check the zip	
28. Fit the battery and switch on – check the lights they should cycle through and stop on green – TURN OFF	
29. Remove small plastic caps from filters and screw into place – ensure they “lock” into place. – THE LARGE CLEAR PLASTIC COVER MUST REMAIN IN PLACE if necessary remove the film underneath and reapply the plastic cover.	
30. Turn on the unit and block the top of the air hose (inside the hood) and check that the red light is illuminated, remove hand and allow to cycle back through to stop on green.	
31. The suit is ready for use only if the green light illuminated. Discard if amber or red and ask the Decontamination Team Leader to LOG the that the suit was not usable.	
32. Prepare to get the next group ready in 45 min.	
33. IF WET Decontamination, ASSIST DECON NURSE with DOFFING:	
34. Inside each of the Blue PRPS Boxes, you will find a bag – place this bag open and facing upwards so the PRPS wearer can stand inside it	
35. Before removing the PRPS suits, ensure a pH test has been completed and pH note of EACH suit is clearly made. IF the pH is acidic or alkaline, ensure a BLACK mark is added on the PRPS Boxes and proceed with caution.	
36. Wearer to stand inside the bag and spread arms out into star jump pose	
37. PPE Support staff to unzip the suit	
38. Wearer to withdraw their hands and arms from the sleeves but keep inside the suit (cross arms over chest)	
39. Wearer to duck forward as the buddy pulls the hood back and off their head	
40. Unclip the AFU and switch off	
41. PPE Support to roll suit down into the bag avoiding any contact between the outside of the bag and the suit	
42. When able and safe, the wearer is to step out of the suit	
43. PPE Support Staff to close the bag and seal with cable tie in the box	
44. Fill in the necessary details on the tag	
45. Place suits on top of each other outside of the decontamination room in piles: IF any boxes have been marked with a BLACK, these need to be piled separately.	
46. Notify Porters that ‘ USED PRPS Suits are ready for collection ’	

Staffing

If this plan is activated, any requests for staffing will be escalated through the CMG Tactical Advisor. Where issues cannot be resolved within the CMG, this will be escalated to the Tactical Commander to manage and/or consider requesting mutual aid via the UHL Strategic Commander.

Actions upon stand-down of this plan:

The decision to stand-down this plan will be communicated via Section 5.11 of the Incident Response Plan

1. Attend Debrief

2. Keep all paperwork, notes, records and logs associated with the incident and at the end of the incident, submit these to the **EPRR Team**

Role Continuity

1. Only undertake this role if you are fit and well, and adhere to working time limits

2. If the incident extends past your shift, arrange a suitable replacement and allow time for handover

Communications

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